

Surgery for vaginal cancer

Surgery to treat vaginal cancer is usually only used for small stage 1 cancers and for cancers not cured by radiotherapy. Surgery for vaginal cancer is done by a specialist gynaecological surgeon.

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Surgery for vaginal cancer

There are different types of surgery for [vaginal cancer](#) depending on the [stage](#) and where the cancer is in the vagina.

Surgery to treat vaginal cancer (vaginectomy) is usually only used for stage 1A cancers and for cancers not cured by [radiotherapy](#).

Some people decide to have surgery instead of radiotherapy because they would like to get pregnant in the future.

The type of surgery you have will depend on:

- where the cancer is in the vagina
- how far it has grown.

Before having surgery, your surgeon and specialist nurse will explain what it involves. You may need some tests before surgery to make sure you are well enough. These are usually done at a pre-operative assessment clinic.

Removing part or all of the vagina (vaginectomy)

Some people may need to have the upper part of the vagina removed (partial vaginectomy), or all of the vagina removed (radical vaginectomy).

During a vaginectomy, the surgeon will remove the cancer and some healthy tissue around it so they can check it has not spread.

If you are having a partial vaginectomy, your surgeon will repair the place where the cancer is removed.

If all the vagina is removed, a specialist plastic surgeon can sometimes make a new vagina (vaginal reconstruction) using tissue from other parts of the body. Vaginal reconstruction surgery is complicated and can have several side effects. The result may not be successful. For these reasons, vaginal reconstruction is rarely available in hospitals in the UK.

Vaginectomy and hysterectomy

Your surgeon may need to remove some other nearby organs as well as the vagina. They may advise removing:

- the womb – this is called a [hysterectomy](#)
- both ovaries and fallopian tubes – this is called a bilateral salpingo-oophorectomy or BSO
- lymph nodes close to the womb (pelvic nodes)

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- lymph nodes higher up in the abdomen (para-aortic nodes)
- the omentum – a layer of fat and tissue covering the organs in the front of the tummy area (abdomen).

Lymph nodes in the abdomen and pelvis

Image: The image shows a woman's lower abdomen, pelvis and the top of the legs from the front. A narrow opening is shown between the legs. This leads into a narrow space shown inside the pelvis. This space is called the vagina. Above the vagina the space narrows until both sides almost touch. Then it opens again to form a hollow, pear-shaped organ called the womb. The surfaces of the two sides are highlighted where they narrow between the vagina and the womb. The highlighted area is the cervix. At either side of the upper end of the womb are two small, solid oval shapes. These are the ovaries. They are connected to either side of the womb by narrow tubes called the fallopian tubes. A network of fine lines connecting small, oval shapes is shown. This is the lymphatic system and lymph nodes. This network enters the image just above the tummy button. The lymph nodes at this level are the para-aortic lymph nodes. The network then splits in to two main streams. These streams are shown sitting over each ovary, down each side of the womb and into the area each side of the vagina at the top of the legs. These are the pelvic lymph nodes.

Types of surgery

This surgery can be done in different ways:

Keyhole (laparoscopic) surgery

Keyhole (laparoscopic) surgery is when the surgeon makes several small cuts in the tummy (abdomen) instead of a large single cut. This leaves small wounds, so you usually recover faster. The surgeon puts a thin tube with a light and camera on the end through one of the small cuts in the skin. This tube is called a [laparoscope](#). It allows the surgeon to examine the area and remove tissue through the cuts.

Keyhole robotic surgery

Keyhole robotic surgery is when keyhole surgery is helped by a machine. Instead of the surgeon holding the laparoscope and surgical equipment, they are attached to robotic arms. The surgeon controls the robotic arms, which can move very precisely and accurately.

Abdominal (open) surgery

Abdominal (open) surgery is when the surgeon makes one cut in the tummy (abdomen). After this surgery, you have a wound that goes across your tummy just above the hips. Or a wound that goes down from the belly button to just above the hips.

Further surgery

During surgery, the surgeon will check nearby lymph nodes and other organs to find out more about the [stage](#) of the cancer.

Removing lymph nodes

The surgeon may remove lymph nodes to check for cancer cells. These may be nodes:

- close to the vagina (pelvic nodes)
- higher up in the abdomen (para-aortic nodes).

Nodes in the groin may sometimes be removed if the cancer is lower down in the vagina.

If pre-operative scans show the cancer may have spread further, they may remove more lymph nodes during the surgery.

Your surgeon will talk to you about the possible benefits and disadvantages of removing lymph nodes.

Surgery if the cancer has spread

If the cancer has spread further through the pelvis, sometimes a major operation can be done to remove this. This surgery is called a [pelvic exenteration](#). This surgery also removes the bladder and bowel.

If the cancer has spread to the liver or lungs, surgery is not usually possible. Rarely, surgery can be done to remove a secondary cancer. But only if it is in one area, with no signs of cancer anywhere else.

These types of surgery can help control the cancer and help make other treatments more effective.

After your operation

How quickly you recover from surgery for vaginal cancer will depend on the type of operation you have and the extent of the surgery.

When you wake up you may have some tubes attached to your body. You may have an intravenous infusion (IV drip) of fluids into your vein. During surgery, you may have a small, plastic tube placed into your tummy (abdomen) or groin. This is to drain any lymph fluid that collects in that area. You will have a small, flexible plastic tube put into your bladder to drain urine (pee) into a bag. You may have a dressing in your vagina (vaginal pack). This will be removed the next day.

It is normal to have some pain or discomfort after surgery. If you had laparoscopic (keyhole) surgery, the gas in your tummy can also make you feel uncomfortable for a few days. The feeling goes away as your body absorbs the gas.

Your healthcare team will want to get you up and moving as soon as possible after surgery to help your recovery. So, it is important that you are comfortable. You will be given painkillers to help but tell your nurses if you are feeling pain.

Your feelings

You may have all kinds of [emotions](#) after your surgery. These feelings may start to get better after a few days, or you may continue to feel this way. Tell your doctor or specialist nurse if you are not feeling better or if you begin to feel very low or [depressed](#). You may need extra [help and support](#).

Some hospitals have local [support groups](#) for people who have gynaecological cancers. You may find a support group helpful. Ask your nurse specialist if there is one in your hospital or local area.

Booklets and resources



Booklets

01 Nov 2023

How are you feeling? The emotional effects of cancer



Booklets

01 Nov 2023

Your feelings after cancer treatment

Going home

If you have laparoscopic or robotic surgery, you can usually go home later the same day, or the next day. If you have an abdominal (open) surgery, you can usually go home 2 to 5 days after the operation.

You may go home with the catheter to help with passing urine (peeing) as you heal. This can usually come out after a few days.

Your nurse will give you advice on:

- your recovery
- how to care for your wounds
- how to care for your catheter, if you have one.

How quickly you recover will depend on the type of surgery. It is important not to do too much for a while. Try to get plenty of rest, do gentle exercise and eat well. If you have any problems, it is important to contact your doctor or specialist nurse.

We have more information about [recovering from a hysterectomy](#) including about physical activity and driving after surgery. It also has information about sex.

Surgery for vaginal cancer can sometimes affect stimulation and sensation during sex. We have more information about cancer treatment and your [sex life](#).

Your doctor or nurse can give you advice about ways you can prevent or cope with vaginal changes. Try to follow their advice and ask for more help if you need to. It can be hard to talk about these side effects, but your cancer team can usually help.

Follow-up

You will be given an appointment to an outpatient clinic to visit the surgeon. You may also meet with your specialist nurse. Instead of going to the hospital, the appointment might be over the telephone.

Your surgeon or nurse will ask about how you are feeling. They will ask questions so they can make sure you are recovering as expected. They will explain the results of your surgery. If any further treatment is needed, they can talk to you about this. Some people have [radiotherapy](#) or [chemoradiation](#) after surgery to help prevent the cancer coming back (adjuvant treatment).

Possible long-term complications

Most people do not have any long-term complications after surgery. But having other cancer treatments as well as surgery can increase the risk.

If your pelvic lymph nodes were removed, there is a risk of developing swelling (lymphoedema) in one or both legs. This is a build-up of lymph fluid in the tissues. [Lymphoedema](#) is not common. But if you have radiotherapy as well as surgery, there is a higher risk.

Getting support

It is common to feel anxious after surgery. You may feel your recovery is taking longer than you expected. Or you may be worried about having further treatment. It is often helpful to talk about your feelings with your family and friends. You can also talk to your specialist nurse or you can call our cancer support specialists on [0808 808 00 00](#).

About our information

This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer.

References



Below is a sample of the sources used in our vaginal cancer information. If you would like more information about the sources we use, please contact us at informationproductionteam@macmillan.org.uk

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Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

How we can help

Clinical Information Nurse Specialists

Our Cancer Information Nurse Specialists are dedicated cancer nurses available to talk to on our Macmillan Cancer Support Line.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

An anonymous network of people affected by vaginal cancer which is free to join. Share experiences, ask questions and talk to people who understand.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help